SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

Cetuximab · every two weeks

Colorectal cancer · NCCP 00732 · version 2 · 14-day cycle

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Patient name	DOB	Hospital no.	Date

Treatment context
Cetuximab alone is an option for RAS wild-type bowel cancer that has spread after oxaliplatin and irinotecan have failed, when irinotecan cannot be tolerated. Your team explains the aim, alternatives and how benefit will be reviewed.

Schedule
Cetuximab on Day 1. No home chemotherapy pump. A repeating 14-day cycle. The number of cycles depends on benefit, side effects and your treatment plan.

1. GENERIC SACT RISKS / DISCUSSION POINTS

- ☐ Purpose, expected benefit and treatment plan explained
- ☐ Treatment may be delayed, modified or stopped because of toxicity
- ☐ Unexpected, rare or serious adverse effects can occur
- ☐ Supportive medicines and urgent contact advice discussed
- ☐ Pregnancy avoidance / contraception / fertility discussed where relevant
- ☐ Bone-marrow suppression / infection risk
- ☐ Anaemia / fatigue and low platelets / bleeding
- ☐ Nausea / vomiting and bowel disturbance
- ☐ Mouth soreness / mucositis and skin changes
- ☐ Allergic / infusion reaction and other organ-specific toxicity

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Cetuximab
EGFR-TARGETED ANTIBODY

- ☐ Rash, dry skin, cracks and sore nails
- ☐ Eye inflammation or corneal injury
- ☐ Low magnesium, calcium or potassium
- ☐ Diarrhoea, dehydration and kidney problems
- ☐ Infusion reaction; rare serious lung or skin effects

Discuss the treatment aim, alternatives, expected benefit, serious and lasting risks, fertility/pregnancy precautions and the contact plan. Confirm RAS selection for colorectal treatment, infusion premedication, skin care and post-treatment mineral checks.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00732 v2.

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